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Makary's successor faces tough task managing FDA amid budget cuts

The Trump administration's recent cuts at the FDA have put the agency's executive capacity under strain, which may lead to medicine shortages and less innovation. Marty Makary's successor as the new FDA commissioner thus faces a tough task

Marty Makary at a news conference earlier this year

Executive summary

Marty Makary, the US Food and Drug Administration (FDA) commissioner, has been ousted just over a year into the role amid criticism from both inside and outside the Trump administration, over his stance on flavoured vapes and abortion pills, among others.

Makary's yet-to-be-confirmed successor faces a challenging task in maintaining drug supply and fostering innovation amid significant budget cuts from the Administration.

Why the FDA’s executive capacity matters

The executive capacity of the US Food and Drug Administration (FDA) is critical. It proactively monitors drug supply chains, which can nip potential manufacturing issues and medicine shortages in the bud.

The agency also ensures the timely review of new drugs and medical devices, thereby balancing innovation with rigorous safety standards.

The FDA’s ability to balance speed with safety helps explain why the US has a more innovative pharmaceutical sector than Europe. It approves new treatments more quickly while still maintaining rigorous safety standards. In short, the FDA is critical to preventing drug shortages and fostering innovation.

Yet the Trump administration recently proposed a roughly $7bn (5.5%) cut to the HHS budget for the 2026 fiscal year, and a 3.9% cut (or $271 million) to the FDA's budget, which lowered the FDA’s headcount. Both cuts have reduced the FDA’s executive capacity, increasing the risk of drug shortages and undermining the US pharmaceutical sector's innovative capacity.

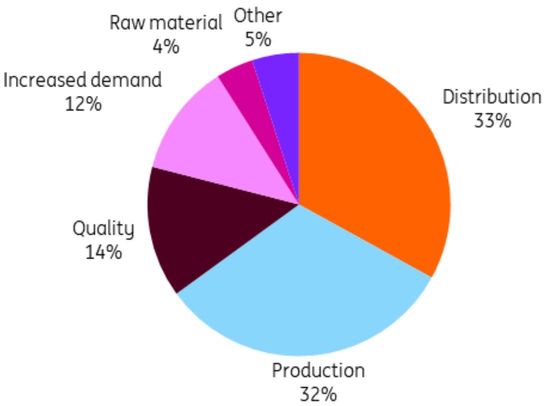
FDA cuts risk increasing drug shortages

When FDA inspections of manufacturing facilities and drug supply chains become less frequent, incentives for preventive maintenance, quality staffing, and internal controls decrease, especially when plant managers are under pressure to meet production and cost targets.

Over time, this may lead to manufacturing problems that disrupt production and increase the risk of medicine shortages. Especially because production and distribution together account for roughly two-thirds of medicine shortages.

Distribution and production account for two-thirds of medicine shortages

Reasons for drug shortages in percentages, 2023



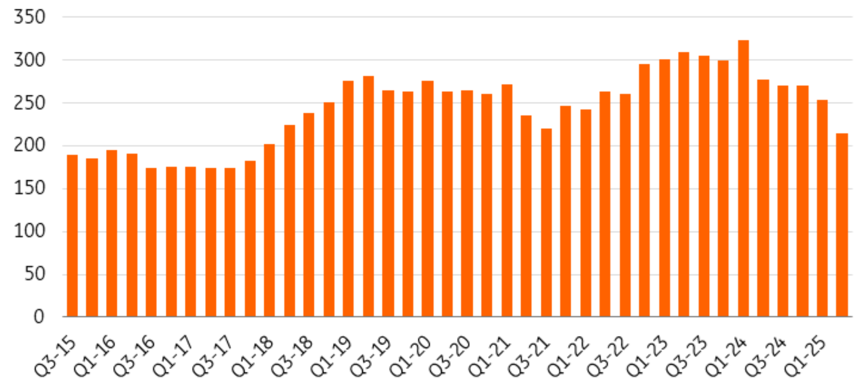
Source: KNMP; ING

New research from Tilburg University and ING finds that when an FDA inspection is delayed, the risk of a drug shortage rises sharply for any product made in a facility that is inspected late, or not inspected at all. This is particularly true for high-risk medicines (i.e. medicines that are clinically critical and complex to manufacture).

High-risk medicines (such as cell/gene therapies) are correlated with a 2.5 times higher risk of shortage when an FDA inspection runs late, with shortages often peaking nine to 15 months after a plant's delayed inspection. The correlation is particularly strong for injectables, which have a roughly 3.5-fold higher risk of shortage. By undermining the FDA's executive capacity, the US administration risks exacerbating drug shortages, which are already elevated globally due to fragile supply chains.

US drug shortage remains high

Active US drug shortages by quarter



ASHP; ING

FDA cuts also put the innovative capacity of the US at risk

The FDA budget cuts could also threaten the sector's innovative capacity. The Government Accountability Office (GAO) reported last month that employee churn, hiring challenges and investigator attrition have reduced inspection capacity, including the Bioresearch Monitoring (BIMO) programme.

BIMO inspections assess the conduct of clinical trials, data reliability, and participant protection. Training investigators takes two to three years. Fewer inspections may weaken oversight, slow approvals, and delay medicine launches, which has historically been an edge for the US. In the same month, the FDA shifted Phase 3 approvals from two trials to a single, confirmatory-evidence-supported default. This can potentially speed up access but increase reliance on a single study. Meanwhile, strained inspection support hampers plant inspections that safeguard quality and prevent shortages.

In short, the cuts at the FDA may delay time-to-market in the US and compromise drug safety (particularly in therapeutic areas such as psychiatric drugs). But most importantly, they threaten the FDA's predictability, thereby adding volatility for drugmakers. This, in turn, is bad for investment in a risky sector such as biotech and could hamper innovation in the medium run.

It's no wonder, then, that hundreds of industry leaders signed a letter to now former FDA Commissioner Marty Makary about the importance of a predictable regulatory agency. They cited a survey in which 82% of biopharma respondents said they were concerned about the FDA's ability to function.

Makary's successor should keep the FDA focused on execution

FDA drug approvals remain broadly on time, and the agency retains many excellent people. For Makary's successor, the priority now is to keep the machine running: retain and empower experienced staff, keep reviews predictable and transparent for industry, and triage inspection and shortage-prevention work toward the highest-risk products and facilities. The budget cuts are worrisome – and the most practical response is a disciplined focus on execution that protects both supply reliability and the FDA's credibility.

A very special thanks goes to Jurgen Vermeulen for his help with this report.

Author

Diederik Stadig
Senior Economist, Healthcare & Technology
diederik.stadig@ing.com

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